Disease Outlook and Preparedness Briefing

Demo disease

Prepared: 2026-10-13 22:31 PHT

Context: Synthetic / Demo Data

Location: Not specified

Population: all-age

Case classification: Not specified

Source: built-in demo

Source date: Not specified

Latest complete week: 2025 W52

Forecast issued: 2026-10-13T00:00:00Z

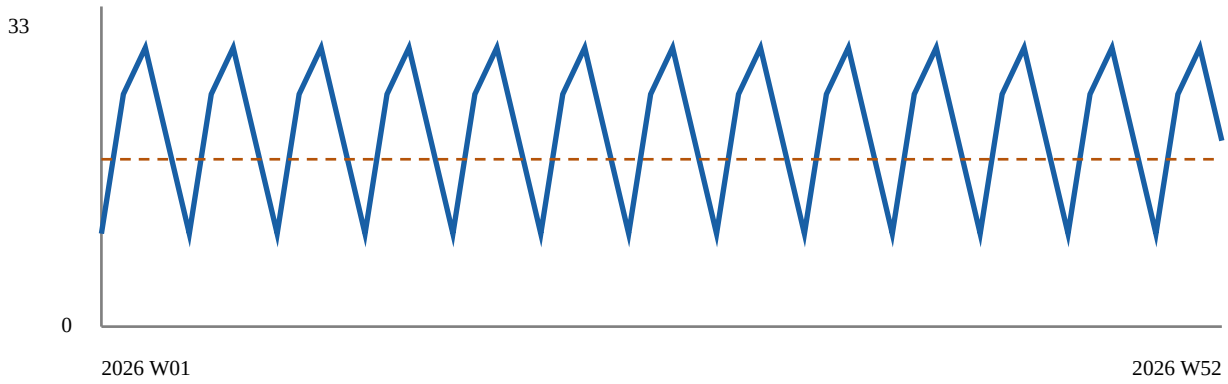
Model protocol: Not specified

Outlook: 52 weeks | Baseline: 18 cases

Executive summary

39 of 52 projected weeks exceed the latest complete week (2025 W52: 18 cases). The projected total is 1,105.0 cases, with a peak of 30.0 in 2026 W03. The first above-baseline week is 2026 W02.

Weekly outlook



Blue: projected weekly cases. Dashed amber: latest complete week baseline. Values below are rounded for display.

Week	Projected cases	Case change	% change	Comparison
2026 W01	10.0	-8.0	-44.4%	Below baseline
2026 W02	25.0	+7.0	+38.9%	Above baseline
2026 W03	30.0	+12.0	+66.7%	Above baseline
2026 W04	20.0	+2.0	+11.1%	Above baseline
2026 W05	10.0	-8.0	-44.4%	Below baseline
2026 W06	25.0	+7.0	+38.9%	Above baseline
2026 W07	30.0	+12.0	+66.7%	Above baseline

Week	Projected cases	Case change	% change	Comparison
2026 W08	20.0	+2.0	+11.1%	Above baseline
2026 W09	10.0	-8.0	-44.4%	Below baseline
2026 W10	25.0	+7.0	+38.9%	Above baseline
2026 W11	30.0	+12.0	+66.7%	Above baseline
2026 W12	20.0	+2.0	+11.1%	Above baseline
2026 W13	10.0	-8.0	-44.4%	Below baseline
2026 W14	25.0	+7.0	+38.9%	Above baseline
2026 W15	30.0	+12.0	+66.7%	Above baseline
2026 W16	20.0	+2.0	+11.1%	Above baseline
2026 W17	10.0	-8.0	-44.4%	Below baseline
2026 W18	25.0	+7.0	+38.9%	Above baseline
2026 W19	30.0	+12.0	+66.7%	Above baseline
2026 W20	20.0	+2.0	+11.1%	Above baseline
2026 W21	10.0	-8.0	-44.4%	Below baseline
2026 W22	25.0	+7.0	+38.9%	Above baseline
2026 W23	30.0	+12.0	+66.7%	Above baseline
2026 W24	20.0	+2.0	+11.1%	Above baseline
2026 W25	10.0	-8.0	-44.4%	Below baseline
2026 W26	25.0	+7.0	+38.9%	Above baseline
2026 W27	30.0	+12.0	+66.7%	Above baseline
2026 W28	20.0	+2.0	+11.1%	Above baseline
2026 W29	10.0	-8.0	-44.4%	Below baseline
2026 W30	25.0	+7.0	+38.9%	Above baseline
2026 W31	30.0	+12.0	+66.7%	Above baseline
2026 W32	20.0	+2.0	+11.1%	Above baseline
2026 W33	10.0	-8.0	-44.4%	Below baseline
2026 W34	25.0	+7.0	+38.9%	Above baseline
2026 W35	30.0	+12.0	+66.7%	Above baseline
2026 W36	20.0	+2.0	+11.1%	Above baseline
2026 W37	10.0	-8.0	-44.4%	Below baseline
2026 W38	25.0	+7.0	+38.9%	Above baseline

Week	Projected cases	Case change	% change	Comparison
2026 W39	30.0	+12.0	+66.7%	Above baseline
2026 W40	20.0	+2.0	+11.1%	Above baseline
2026 W41	10.0	-8.0	-44.4%	Below baseline
2026 W42	25.0	+7.0	+38.9%	Above baseline
2026 W43	30.0	+12.0	+66.7%	Above baseline
2026 W44	20.0	+2.0	+11.1%	Above baseline
2026 W45	10.0	-8.0	-44.4%	Below baseline
2026 W46	25.0	+7.0	+38.9%	Above baseline
2026 W47	30.0	+12.0	+66.7%	Above baseline
2026 W48	20.0	+2.0	+11.1%	Above baseline
2026 W49	10.0	-8.0	-44.4%	Below baseline
2026 W50	25.0	+7.0	+38.9%	Above baseline
2026 W51	30.0	+12.0	+66.7%	Above baseline
2026 W52	20.0	+2.0	+11.1%	Above baseline

Interpretation by period

2026 W01 to 2026 W04: 85.0 projected cases; 3 of 4 weeks above baseline.

2026 W05 to 2026 W08: 85.0 projected cases; 3 of 4 weeks above baseline.

2026 W09 to 2026 W12: 85.0 projected cases; 3 of 4 weeks above baseline.

2026 W13 to 2026 W16: 85.0 projected cases; 3 of 4 weeks above baseline.

2026 W17 to 2026 W20: 85.0 projected cases; 3 of 4 weeks above baseline.

2026 W21 to 2026 W24: 85.0 projected cases; 3 of 4 weeks above baseline.

2026 W25 to 2026 W28: 85.0 projected cases; 3 of 4 weeks above baseline.

2026 W29 to 2026 W32: 85.0 projected cases; 3 of 4 weeks above baseline.

2026 W33 to 2026 W36: 85.0 projected cases; 3 of 4 weeks above baseline.

2026 W37 to 2026 W40: 85.0 projected cases; 3 of 4 weeks above baseline.

2026 W41 to 2026 W44: 85.0 projected cases; 3 of 4 weeks above baseline.

2026 W45 to 2026 W48: 85.0 projected cases; 3 of 4 weeks above baseline.

2026 W49 to 2026 W52: 85.0 projected cases; 3 of 4 weeks above baseline.

Recommendations and suggested intervention plan

Action 1

Finding: First projected increase: 2026 W02.

Activity: Review the latest complete reports, reporting delays and local case information before adapting preparedness activities.

Proposed team: City health officer / CESU

Timing: At report review; repeat when new weekly data arrive

Follow-up indicator: Weekly records reviewed and discrepancies followed up.

Basis: Application planning suggestion

Action 2

Finding: No verified disease-specific guidance is configured.

Activity: Ask the relevant disease program team to identify applicable DOH/local guidance before selecting disease-specific interventions.

Proposed team: City health officer / disease program team

Timing: Before issuing disease-specific recommendations

Follow-up indicator: Applicable guidance identified.

Basis: Application planning suggestion

Evidence and limitations

Projected increases are comparisons with one observed week, not outbreak declarations or verified alert-threshold crossings.

The outlook begins after the model data cutoff, not necessarily the current calendar week. Confirm data freshness before operational use.

Forecasts do not establish causes, barangay hotspots or individual clinical decisions. Lower projections do not exclude an outbreak.

Intervention timing, proposed teams and follow-up indicators are application planning suggestions, not DOH mandates. No Antipolo-specific protocol has been verified.

Any displayed model range is provisional, not a validated confidence interval. Longer outlooks should be reassessed as new observations arrive.

population: requires established 5–19; found ['all–age'].

case_classification: requires established confirmed; found unknown.

location: requires established Antipolo City; found unknown.

dataset_type: requires established real; found ['synthetic'].

Source metadata incomplete: CESU/PIDSAR provenance and source file required.

Disease scope awaits documented approval.

Sufficient history cannot be established until the weekly protocol is approved.

References

No verified disease-specific reference configured.